

Encorafenib (Braftovi®) and cetuximab (Erbix®)

Encorafenib (Braftovi®) and cetuximab (Erbix®) is a cancer drug treatment. It is used to treat bowel cancer.

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What is encorafenib (Braftovi®) and cetuximab (Erbix®)?

Encorafenib (Braftovi®) and cetuximab (Erbix®) is a treatment that combines 2 targeted therapy drugs. This treatment may be used to treat advanced bowel cancer. It is used if [tests](#) show that the cancer cells have both of the following:

- a normal RAS gene
- a gene change called a BRAF V600E mutation

Encorafenib is a type of targeted therapy drug called a cancer growth inhibitor. Cetuximab is a type of targeted therapy called a monoclonal antibody.

It is best to read the information about this drug with our general information about [targeted therapy drugs](#) and the [type of cancer](#) you have.

Your cancer doctor will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How encorafenib and cetuximab are given

You usually have this treatment at a day unit or clinic as an outpatient. Cetuximab is given as a drip into a vein (intravenous infusion). Encorafenib is taken as capsules that you swallow.

During treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your targeted therapy.

Having cetuximab as a drip

You have cetuximab as a drip into a vein (intravenous infusion). The nurse will run the infusion through a pump. This will give you the treatment over a set time.

You have cetuximab through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in your arm or hand
- a [central line](#) – a fine tube that goes under the skin of your chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in your arm and goes up into a vein in your chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on your chest or arm and goes into a vein in your chest.

You have the first infusion over 2 hours and then stay in the clinic for about 1 hour after. This is to check you do not have a reaction to the drug. If you do not have any problems, your next treatment can be given at faster rate depending on dose.

Taking encorafenib capsules

You take encorafenib as capsules. Your cancer team will give you them to take home. They may give you capsules of different strengths.

When you take encorafenib capsules make sure you:

- swallow them whole with a glass of water
- do not chew, open or crush them
- take them at the same time every day.

You can take encorafenib with or without food. Avoid eating grapefruit or drinking grapefruit juice when taking encorafenib. They may make the side effects worse.

- If you forget to take the capsules and it is:
- **more than 12 hours** until your next dose, take the missed dose as soon as you remember.
- **less than 12 hours** until your next dose, wait and take the next dose at your usual time. Do not take a double dose.

Other things to remember about your capsules:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, somewhere children cannot see or reach them.
- If you are sick just after taking the capsules or tablets, contact the hospital. Do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused capsules. Return them to your cancer team at the hospital.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. You should take all your capsules or tablets exactly as they have been explained to you. This is important to make sure they work as well as possible for you.

Your course of treatment

Your doctor, nurse or pharmacist will discuss your treatment plan with you.

You usually have a course of several cycles of treatment over a few months. Each cycle lasts 28 days. You take encorafenib once a day every day. You usually have cetuximab on day 1 and day 15.

You can continue to have encorafenib and cetuximab for as long as it is working for you and if the side effects are manageable.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Some people may have side effects while they are being given this treatment or shortly after they have it.

Allergic reaction

Some people have an allergic reaction to cetuximab. This can happen while having the treatment or for up to 1 hour afterwards. You will be given drugs before the infusion to reduce the chance of a reaction.

A reaction is usually mild but, rarely, can be more severe. If you have a reaction, the nurses can usually treat it by slowing or stopping the drip. Then you will have the rest of your treatments more slowly.

Your nurse will check you for signs of a reaction during and for a while after your treatment. Always tell your them straight away if you feel unwell or have any of the following symptoms:

- feeling hot or flushed
- a skin rash
- itching
- shivering
- feeling dizzy
- a headache
- feeling breathless
- swelling of your face or mouth
- pain in your back, tummy or chest.

Rarely, people may get a reaction a few hours after treatment. If you have any of the symptoms above or feel unwell after you get home, contact the hospital straight away on the 24-hour number. Always call 999 if swelling happens suddenly or you are struggling to breathe.

You are more likely to have a serious allergic reaction to cetuximab if you have an allergy to:

- red meat
-

tick bites - a tick is a tiny spider-like insect found in woodland and heath areas.

You must tell your doctor and nurse if you have either of these allergies before they give you cetuximab.

Some people have an allergic reaction to encorafenib. The symptoms are similar to those we describe in an allergic reaction to cetuximab. If you feel unwell or have any of these signs, contact the hospital straight away.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

[If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Difficulty sleeping (insomnia)

Sometimes this treatment can affect your sleep. Tell your doctor, nurse or pharmacist if cancer treatment makes it [difficult to sleep](#).

Feeling tired

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Skin changes

This treatment can cause an acne-like rash on the face, neck, chest and back. The rash is most likely to start in the first 2 or 3 weeks of treatment. Your skin may also feel dry, itchy, scaly or tender.

[Skin changes](#) are often mild but can be more severe in some people. If you notice any skin changes, contact the hospital as soon as possible on the 24-hour number you have been given. You may need creams, steroids or antibiotics. Your doctor may stop your treatment until the skin changes improve.

Here are some things you can do to take care of your skin during treatment:

- Wash with lukewarm water and mild, unperfumed, soap-free cleansers.
- Do not use products containing alcohol on your skin, or anti-acne products (unless they have been prescribed).

- Moisturise your skin regularly with unperfumed moisturisers for dry skin conditions.
- Use sun cream of at least SPF 30 to [protect your skin from the sun](#). Cover up with clothing and a hat.

Skin changes usually improve when treatment ends.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Bleeding

This treatment may increase your risk of bleeding. Contact the hospital on the 24-hour number you have been given, if you have any bruising or bleeding that you cannot explain. This includes:

- nosebleeds
- blood in your pee (urine) or poo (stools)
- bleeding from the back passage (rectum).

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called [peripheral neuropathy](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug. The symptoms usually improve slowly after treatment ends. But for some people they continue and are a long-term side effect of treatment.

Muscle or joint pain

You may get pain in your muscles or joints for a few days after treatment. If this happens, tell your doctor, nurse or pharmacist. They can give you painkillers and advice. They can also tell you if any of the painkillers you usually take are suitable.

Tell them if the pain does not get better. Having warm baths and resting regularly may help.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about [coping with changes to taste](#).

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Serious skin changes

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital. Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Hair changes

This treatment may cause hair changes. You may have some gradual hair loss a few months after treatment starts. You will not usually lose all the hair on your head.

You might also notice:

- a change in the texture or colour of your hair
- more hair growth on your face or chest
- your eyelashes growing longer, or your eyebrows getting thicker.

If you are having problems with [hair changes](#), ask your cancer team for advice. Do not try to cut your eyelashes yourself. Your cancer team can arrange for an eye doctor to do this.

Any hair changes usually go back to normal after treatment ends.

Liver and kidney changes

This treatment may cause changes in the way your liver and kidneys work. Your doctor or nurse will take regular blood samples to check how well your liver and kidneys are working. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids also helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Skin cancer risk

This treatment can increase your risk of developing skin cancer. Your doctor, nurse or pharmacist can explain this. Treating the colorectal cancer will usually outweigh the risk of this. It is important to get any new skin changes, or changes to a mole checked during or after treatment.

Sore eyes

This treatment may make your eyes dry, sore, red and itchy. Your doctor, nurse or pharmacist can prescribe eye drops to help with this.

Contact the hospital on the 24-hour number if:

- your eyes do not improve after taking the eye drops
- you have changes to your vision, such as blurred vision or any sight loss
- you have pain in your eyes or they feel sensitive to light.
- your eye or eyelid becomes red or inflamed or feels swollen.

Effects on the heart

This treatment can affect how the heart works. You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
-

use illustrations to explain text

- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)